

إسم المريض رابعياً : فاطمه عمران محمد الفولى

المحافظة : الجيزة الحي / المدينة : بولاق الدكرور العنوان : 18

رقم البطاقة : 26710110100308 رقم التليفون : 01154605595 تاريخ تسجيل الإستمارة : 23-09-2026

العمر(تاريخ الميلاد) : 11-10-1967 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطه العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثالث اكسيمستان **قسم العلاج :** الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

التوقيع: 

التوقيع:

التوقيع: (مستند رقم ١٢٣٤٥٦٧٨٩٠)

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشئون الصحية

رئیس

المجلس الطبي المتخصص

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Name: فاطمه عمران محمد القبول National ID: 26710110100308	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Exemestane hormonal therapy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

فاطمة
عمران محمد الفولي

١٨ ش حسين محمد حسين ش فاهيا
بولاى الدكرور - الجيزة

١٩٦٧/١٠/١١ ٢٦٧١٠١١٠١٠٠٣٠٨

KE7610392

Dms
مستوى

٢٠٢٣/١١/١١ ٢٦٧٨٠١١٠١٠٠٣٠٨

ربة منزل

أنفى
مستلمة
مسترجعة

مصطفى فؤاد حافظ محمود

البطاقة سارية حتى ٢٠٢٠/١٠/١٧

MDT FORM ٢٥ / ١ / 2026 G

Name	فاطمة احمد ص	MRN	٥١١٥٤٦٥٥٥٥٥
Age	٥٨	ID	3806/٢٥٢٥
Menopausal status	Post M	Phone	٢٢٧١٠١١٠١٠٢٠٨
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	-ve for h	REASON OF MDT	

Clinical Data

BX: adjuvant

ER: 2H PR: 4H HER 2: -ve Ki67:

Sonomammography:

Mets Workup:

2020 → BS → ECU → FORU
→ AE like bar

11/15 → CT → Splenic
+ 2x2 LNS + Adjuvant

Recommendations:

BS → Metastasis, GATA3 + e
Suggesting Breast

In CDK16 → Sub lex approval
٢٥ / ١ / ٢٦

MDT OUTCOME & PLAN

for Bone Scan
then - Fastodex / Palbociclib

SIGNATURE

MEDICAL ONCOLOGY SURGERY RADIOLOGY PATHOLOGY Nutrition Psychiatric

Signature



وحدة دلالات الأورام
قسم الباثولوجيا - قصر العيني
ت : ٠١٠٦٨٥٢٩٦٨٤

IMMUNOHISTOPATHOLOGY

Patient Name: فاطمة عمران محمد lab.No:2025-1592-1542

Date:7/12/ 2025

Age: 58Y

Sex: Female

Markers: ER, PR, Her 2 neu and Ki67 .

Procedure:

Sections were prepared from paraffin blocks treated for (ER-clone SP1), (PR clone 1E2), (Her 2 neu-clone 4B5) and (Ki67- clone 30-9) automatically stained in Bench mark ULTRA (VENTANA) using VIEW DAB Detection Kit, Hematoxylin as counter stain

Results:

Examination of sections treated for ER revealed positive nuclear staining of tumor cells by chromogen.

Examination of sections treated for PR revealed weak nuclear staining in 30% of tumor cells by chromogen.

Examination of sections treated for Her 2 neu revealed no membranous staining of tumor cells by chromogen null type

Examination of sections treated for Ki67 revealed positive nuclear staining in 30 % of tumor cells by chromogen.

DIAGNOSIS: Splenic focal lesion, immunophenotyping:

- POSTIVE FOR ER SCORE (8/8).
- POSITIVE FOR PR SCORE (4/8)
- NEGATIVE FOR HER 2 NEU SCORE(0) NULL TYPE
- KI67: 30%

PROF. DR. SAMIA GABAL
PROF. DR. MONA SALAH

د. سامية غبال
د. منة صلاح

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وحدة دلالات الأورام
قسم الباثولوجيا - قصر العيني

ت: ٠١٠٦٨٥٢٩٦٨٤
IMMUNOHISTOPATHOLOGY

Patient Name: فاطمة عمران محمد

lab.No: 2025-1542

Date: 26/11/2025

Age: 58y

Sex: F

Markers: GATA3, CK7, CK20, TTF1

Procedure:

Sections were prepared from paraffin blocks treated for GATA3, CK7, CK20, TTF1 automatically stained in Bench mark ULTRA (VENTANA) using VIEW DAB Detection Kit, Hematoxylin as counter stain

Results:

Examination of sections treated for GATA3 revealed positive nuclear staining in tumor cells by chromogen.

Examination of sections treated for CK7 revealed positive staining in tumor cells by chromogen.

Examination of sections treated for TTF1 and CK20 revealed no staining of tumor cells by chromogen.

DIAGNOSIS: Splenic focal lesion:

METASTATIC ADENOCARCINOMA OF BREAST ORIGIN

PROF.DR. SAMIA GABAL

PROF.DR. MONA SALAH

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س. ج. ج.

(MT)

Name: Fatma Omran Mohamed

Date: 29/10/2025

R: 37994

Prof. Dr.

Follow Up Post Contrast C.T. Scan Of Chest, Abdomen And Pelvis Revealed:-

- A known case of neoplastic process of left breast underwent surgery followed by chemotherapy and hormonal therapy.
- In comparison with last available CT chest dated 8/9/2025, the current study shows the following data:

I-Chest:

- Still noted with no appreciable changes reaching the size of and number of the previously noted few subpleural pulmonary nodules seen at the posterior segment of left lower lung lobe, the anterior and posterior segments of right lower lung lobe. The largest nodule measures about 3 cm in diameter.
- No pleural effusion.
- Multiple enlarged heterogeneously enhanced hilar and mediastinal LNs are noted. The largest LN seen at pre vascular region measuring about 6.5X4 cm in diameter.
- Average cardiac size with no pericardial collection.

OPINION:-

- Stationary course since last study dated 8/9/2025.

II- Abdomen And Pelvis:

- Enlarged fatty liver with no definite focal lesions or dilated IHBRs.
- Patent main portal vein and its central branches.
- Bulky spleen showing large hypodense focal lesion at its hilum measures about 6.5X5 cm in diameter showing no evidence of contrast enhancement or washout.
- Enlarged left adrenal gland showing soft tissue mass lesion measures about 2X1.5 cm in diameter with faint enhancement.
- A well defined heterogeneously enhanced soft tissue mass lesion is noted at right inguinal region measures about 4X3.5 in diameter surrounded by smudged fat planes and few small nodules and causing focal contour bulge.
- The rest of abdomino pelvic organs showing no abnormality.
- No ascites.

OPINION:-

- Enlarged fatty liver with no focal lesion.
- Bulky spleen with hilar focal lesion? Lymphoma? Other.
- Left adrenal gland mass lesion? Metastasis.
- Right inguinal mass lesion? LN metastasis? Other for biopsy.

Dr. Fatma Naeem

Prof. Dr. Tamer Ahmed Kamal

N.R.



09/16/2026

MDT FORM

Name:	Fatma Omran Mohamed *	MRN:	4429
Age:	59 Y	ID:	26710110100308
Menopausal Sta	POST	Phone: *	01154605595
NRS SCORE:	3	Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:		DP metastatic	
Surgical History: *	negative	ECOG Performance Status	00 10 20 30 4

Clinical Data:

BX: metastatic adenocarcinoma of breast origin

ER: 8/8 **PR:** 4/8 **HER2:** -ve **KI67:**

History:
patient is 58 years old -ve family history 2020, BCS then adjuvant chemotherapy EC X 4 TILL 2025 IN 2025, CT SCAN SHOWED BULKY SPLEEN AND LEFT ADRENAL GLAND MASS AND RIGHT INGUINAL LNs biopsy from spleen: metastatic adenocarcinoma showed GATA3 +ve (suggestive of breast origin) ER: 8/8 PR: 4/8 HER2: -ve +C7 spinal cord compression by extraosseous mass : Received Palliative RTH over cervicodorsal vv then received Faslodex-1brance 2/2026 till 8/2026 then now DP (lung -Liver-Bone)

Sonomammography:

Biomarker/Molecular

Mets Workup:

CT CAP : DP of Lung lesion, HFL Segment VI + 8th rib STM

Recommendations:

For Possibility of re-biopsy from 8th rib STM
Palliative RTH ? (previously received over C7 and now it is the most painful region, too soon for re-Irradiation)
Shift to Aromasine- Affinitor

MDT Outcome & Plan

Shift to Affinitor - Aromasine
Trial of Re-biopsy from STM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. ندى حسن	د. ندى حسن		

R L



TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

Patient's Name:

Fatima Imran Mohammed

Date:

2026-09-02

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported left breast carcinoma, compared to last study dated 25/5/2026, the current study showed:

(I) Chest:

- Left breast minimal skin thickening.
- Mild progressive course regarding the size of the previously seen multiple amalgamated heterogeneously enhancing mediastinal lymph nodes , with the largest seen at right hilar /post mediastinal lymph nodes measuring 7.9 x 7.2 cm .
- Progressive course regarding the size of the previously seen right lung and left lower lobar pulmonary nodules, with the largest measuring 3.5 x 2.2 cm in today's study.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous CT density , with progressive course since last study regarding the previously seen segment IV focal lesion measuring 49 x 35 mm in today's study with the previously seen segment VII focal lesion less appreciated in today's study measures 1.7 x 0.9 cm
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys, aorta and IVC.

Patient's Name:

Fatima Imran Mohammed

Date:

2026-09-02

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

- Bilateral adrenal lesions still seen measuring 2 x 1.6 cm on left side and 2 x 1.3 cm on right side .
- Normal CT appearance of the spleen with progressive course since last study regarding the previously seen large heterogeneously enhancing focal lesion measuring 79 x 44 mm.
- Regular filling of the urinary bladder showing no stones, masses .
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Progressive course regarding the previously seen right 8th rib lytic lesion with extra osseous soft tissue component 43 x 27 mm.
- No vertebral structural collapse .

OPINION:.

- Progressive course regarding the size of multiple right lung and left lower lobar pulmonary nodule as well as the segment IV hepatic focal lesion.
- Mild regressive course regarding the segment VII focal lesion.
- Rather stationary rest of the study.

Dr.sherif Wagdy

Dr.sherif Wagdy

Patient Name: Fatma Emran Mohamed Omar
Prof. Dr :
Patient ID : 1562114

Date 10/09/2026
Age : 59 Year(s)
Sex F

MRI EXAMINATION OF THE CERVICAL SPINE

Technique:

- Sagittal T1, T2
- Axial T1, T2 FFE

Findings:

- Straightened cervical curve
- Reduced vertical height of C4 and C7 showed diffuse infiltrative lesion showed low signal in T1 and T2 with posterior contour bulge more evident at C7 vertebral body seen effacing the anterior subarachnoid space and indenting the cord. Extra-osseous soft tissue component is seen at C7 vertebral body
- Another infiltrative lesion yet with preserved height and boundaries of D1 vertebral body showed low signal in T1 and T2
- No vertebral structural collapse or marrow signal alteration.
- Spondylodegenerative changes of the cervical spine evidenced by:
 - Marginal osteophytic lipping of the opposing vertebral endplates.
 - Reduced height and bright T2 signal of the cervical discs.
- C3-4 down to C6-7 posterior disc bulge/osteophytic lesion seen effacing the anterior subarachnoid space, indenting the cord and encroaching upon the neural exit pathways.
- Normal caliber & MR signal of the cervical cord. No cord edema, malacia or syrinx.
- Intact cervico-medullary junction.

OPINION:

- ✚ C4, C7 and D1 infiltrative lesions with reduced vertical height of C4 and C7 as detailed likely metastatic for bone scan
- ✚ Cervical spondylosis with multilevel disc lesions, as described.

Much Obliged,

Dr. Asser Ahmed, Consultant of Radiology